

Session Objectives

- Discuss the history of the Still technique
- Explain the principles of the Still technique
- Explain the physiologic mechanisms involved
- Discuss the indications and contraindications of the Still technique
- Explain how to apply the principles of Still Technique to treat any given somatic dysfunction based on your knowledge of anatomy and the function of the given region.

History of the Still Technique

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College of Osteopathic
Medicine

- Developed by:

Richard Van Buskirk, D.O., F.A.A.O

Dr. Still didn't teach techniques.

Based on writings of Charles Hazzard, D.O., and descriptions from Carl Phillip McConnell, D.O., and Herb Yates, D.O. who described the method as, "indirect and then direct."



Basic Principles of Still Technique

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- Passive
- Indirect
- Direct
- Muscle Spindle + Articulatory
 - Addresses the Muscular/Myofascial component AND the arthrodiastal component of dysfunction

Contraindications of the Still Technique (1 of 3)

- Patient unable to tolerate treatment positions
 - Modify technique or use a different technique
- Radicular pain when performing to c-spine using compression (similar to Spurling Maneuver)
 - Reposition, try with traction, or try another technique

Contraindications of the Still Technique (2 of 3)

- Any condition of the region being treated where introduction of traction, or compression is contraindicated
 - Hip replacement
 - Shoulder pathology (certain treatment positions for Rib 1)
 - Chronic shoulder dislocation/separation (any treatment that moves and /or puts force into the shoulder)

Contraindications of the Still Technique (3 of 3)

- Recent trauma, fracture or dislocation at area involved with treatment
- Caution (but, not contraindication)
 - Osteoporosis
 - Malignancy
 - Rheumatologic Disorders
 - Congenital malformations
 - Stenosis
 - Other clinical diseases (use your clinical judgement)

Side Effects of the Still Technique

(similar to other osteopathic techniques)

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➤ Transient:

➤ Fatigue

➤ Soreness

➤ Stiffness

Due primarily to the restoration of circulation to previously dysfunctional tissue and release of muscular waste products.

Case

A 50-year-old female physician presents to your office complaining that she twisted her back wrong while treating a rather large patient with muscle energy treatment. She is having a constant sharp pain in the right lower back which is exacerbated with left rotation and deep breathing. Nothing makes it better.

Case (continued)

PMHx: Raynaud's disease

PSx: 2 cesarean sections

Soc Hx: denies alcohol, drugs, tobacco use

Family History: both parents with HTN and 5 siblings alive and well

ROS: Only positive for the back pain

Case (continued)

Plan:

Low Back pain – Hamstring, psoas, Paraspinal mm. stretches, piriformis stretch

Muscle spasm – Stretches as above

Somatic Dysfunction – Lumbar

OMT – Soft tissue, Still Technique

Somatic Dysfunction – Sacrum

OMT – CS

Somatic Dysfunction – Lower Extremity

OMT - MET

Summary

Still Technique:

- Passive, indirect, then direct technique
- Physiologic mechanism - Muscle spindle, articular
- Method – guide to freedoms, compress, maintain compression and guide into and through barrier, return to baseline, reassess